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TO: PT: Green, Louis

FAX: 997074349725

SUBJECT:

Number of pages including this cover sheet: 8

Date and time of transmission: 5/5/2026 8:55:50 AM

FROM:

David Grant Medical Center (DGMC)
101 Bodin Circle
Travis AFB, CA 94535

FAX: 707-423-5055, DSN: 799-5055

Phone: 707-423-5353, DSN: 799-5353

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0014C-David Grant Medical Center
101 BODIN CIRCLE
TRAVIS AFB, CA 94535-

(707) 423-3000

Patient Name:	**Green, Louis O**	Admit:	4/20/2026 15:14 PDT
MRN:	2729165000001	Discharge:	4/20/2026 23:59 PDT
FIN:	201679043	Admitting:	
Sex/DOB/Age:	Male 6/30/1938 87 years	Location:	0014C-FM-FMRC
DOD ID (EDIPI):	1052564707	Veterans ID (ICN):	

Letters

Document Type:	Provider Letter
Service Date/Time:	4/21/2026 12:32 PDT
Result Status:	Auth (Verified)
Perform Information:	MINOR, ANNEKA M (4/21/2026 12:32 PDT)
Sign Information:	

DoD Military Treatment Facility
Defense Health Agency-TRICARE
7700 Arlington Blvd, Falls Church VA
22042

Re: Louis Green
Date of Visit: 04/20/2026

Dear Network Provider,

Below you will find information relevant to the referral order.

Referral Details
Medical Service: Physical Therapy
Referral Reason: OA of rt knee, mobility limited, please evaluate and treat, he is very active
Codified Reason: M25.569 - Pain in unspecified knee
Order Comment:

--This Referral Is Electronically Signed by the Ordering Provider--
Referral Type: Evaluate and Treat (DoD)
Refer from Provider: MARRS, RYAN, DO

This information has been disclosed to you from federal health records in accordance with an approved written request. Unless you are the subject of the record, federal laws may prohibit you from making a further disclosure of this record for a purpose other than why the information was provided to you. If this record contains substance abuse information, it is protected by 42 CFR Part 2 and/or 38 U.S.C. 7332 and the redisclosure restrictions of those federal laws on individuals who are not the subject of the record apply.

0014C-David Grant Medical Center

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Letters

Refer from Location: 0014C-David Grant Medical Center/0014C-FM-FMRC
Referral Written Date: 04/20/2026
Requested Start Date: 04/20/2026
Priority: Routine

Insurance
Health Plan: TRICARE PLUS
Payer: TRICARE
Financial Class: Tricare
Subscriber: Green, Louis O
Member Number: 00193995200
Group Number: 346
Begin Date: 08/04/2025

Other Insurances
Health Plan: 602 Direct Care and TRICARE Mail Order and Retail Pharm
Payer: TRICARE
Financial Class: Tricare
Subscriber: Green, Louis O
Member Number: 00193995200
Group Number: 602
Begin Date: 08/04/2025

Health Plan: SERVICE AFFILIATES
Payer: Self Pay
Financial Class: SERVICE AFFILIATES
Subscriber: Green, Louis O
Member Number: 00193995200
Group Number: M000
Begin Date: 08/04/2025

Health Plan: 000 SERVICE AFFILIATES RX
Payer: Self Pay
Financial Class: SERVICE AFFILIATES
Subscriber: Green, Louis O
Member Number: 00193995200
Group Number: P000

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0014C-David Grant Medical Center

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Letters

Begin Date: 08/04/2025

Additional Referral Information
Patient ID: 1052564707
Patient DOB: 30-JUN-1938
Patient Gender: Male
Patient Phone: 7073728100
Sponsor DOD ID: 1052564707
Case Number: 621367478
Provider NPI: 1992598759
Defer to Network Reason: OHI: other health insurance, .
Instructions to Staff:
Treatment to Date:

Green, Louis O
2009 TOLENAS AVE
FAIRFIELD, CA 945332742

This referral order was electronically signed by the Refer from Provider listed above and is on file in the patient's electronic health record.

Please fax visit reports to the Military Treatment Facility Referral Management Center.

The following document(s) were included in the letter:

Attachment(s): 4/21/2026 12:32 PDT (04/20/2026) Primary Care Note

0014C-David Grant Medical Center
101 BODIN CIRCLE
TRAVIS AFB, CA 94535-

(707) 423-3000

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Office and Clinic Notes

Document Type:	Primary Care Note
Service Date/Time:	4/20/2026 15:14 PDT
Result Status:	Modified
Perform Information:	KLEIN, DAVID A, MD, Family Medicine (4/21/2026 07:48 PDT); MARRS, RYAN, DO (4/21/2026 07:36 PDT)
Sign Information:	KLEIN, DAVID A, MD, Family Medicine (4/21/2026 07:48 PDT); MARRS, RYAN, DO (4/21/2026 07:36 PDT)

Addendum by KLEIN, DAVID A, MD, Family Medicine on April 21, 2026 07:48:24 PDT
I discussed the patient's clinical care with the resident at the time of the visit and agree with the resident's documented findings and plan of care.

David A. Klein, MD, MPH
Col, USAF, MC
Family Medicine Physician / Adolescent Medicine Physician
60 HCOS/SGGF
David Grant Medical Center, Travis AFB, CA

Electronically signed on: 04/21/2026 07:48 PDT

DAVID A KLEIN
MD, Family Medicine

CHIEF COMPLAINT
HLD follow up

ASSESSMENT/PLAN
1. Hyperlipidemia, unspecified
chronic. Lipid panel (4/17/2026): TC: 151, LDL: 98, non-HDL: 102, Trigs: 40; aged out of PREVENT score however w/ pt in relatively great health, lipid management for primary prevention was decided on jt-decision making. Holding patient to 2026 ACC/AHA guidelines for primary prevention for intermediate/borderline 10-yr risk for ASCVD event with LDL-C goal < 100 and non-HDL-C goal < 130; currently meeting goals on statin med and lifestyle choices. Leg pain less likely adverse effect of statin and more likely acute on chronic OA.
- continue fluvastatin 20mg PO daily

PROBLEM LIST/PAST MEDICAL HISTORY
Ongoing
Caregiver annual health check
CKD stage 2
Erectile dysfunction
Essential hypertension
History of left hip replacement
Hyperlipidemia
Osteoarthritis of left hip joint
Osteoarthritis of right hip joint
Overweight
Pain of left hip joint
Prediabetes

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Office and Clinic Notes

- continue diet that includes fibrous vegetables and lean protein, minimal added sugar and salt
- continue with exercise regimen of at least 150 min/wk of moderate activity
- encouraged incorporating vigorous activity (jogging, running, hiking, cycling) for at least 30% of total activity time

2. Knee pain
chronic. patient with rt sided crepitus in max flexion/extension, pain on ambulation with antalgic gait and evidence of tricompartmental OA on recent radiographs (12/2025) is most consistent with acute on chronic OA exacerbation. Given history of b/l hip OA and left TKA, may be secondary to chronic compensation of contralateral leg, leading to more progressive rt versus left knee OA. Low concern for septic jt given lack of effusion, no fevers; low concern for ligamentous/meniscal injury given negative trauma history.
- jt decision making led to PT referral for 4-6 weeks
- turmeric for jt pain approved, advised to take no more than recommended dose
- reassess after this time period if worsening pain, consider CSI in sports med clinic

Ordered:
Referral Request 2.0 - DoD

Ryan N. Marrs, DO
Capt, USAF, MC
Family Medicine Resident Physician, PGY-1
60th HCOS/SGGF
David Grant Medical Center, Travis AFB, CA

Referral Orders - This Visit
Referral Request 2.0 - DoD - Completed
-- 04/20/2026 20:24:00 PDT, Medical Service Physical Therapy, OA of rt knee, mobility limited, please evaluate and treat, he is very active, Evaluate and Treat (DoD), Knee pain

HISTORY OF PRESENT ILLNESS

87 y/o M presents to clinic to discuss HLD and the statin medication that he was put on at his last visit. Pt stated that he wanted to speak with provider to see if it okay for him to take turmeric gummies to support joint health. Pt stated that he would like to discuss muscle defense in clinic today. Pt denies any other concerns in clinic today.

Provider HPI:
I agree with the HPI statement mentioned above with the following addenda:

Patient is a 87 M with hx of well controlled HTN on Eliquis presenting for hyperlipidemia management. reports having switched to fluvastatin after experiencing myopathies and now only feels aching in his rt leg, does not know if it is statin-0induced myopathy or just osteoarthritis.

Additionally has some inquiries on his joint health. Report shaving rt knee pain for quite some time and wanting to start turmeric supplements for management.

REVIEW OF SYSTEMS

Prostate cancer
Prostate cancer
Saddle embolus of pulmonary artery
Sensorineural hearing loss
Spinal stenosis of lumbar region

Historical
No qualifying data

PROCEDURE/SURGICAL HISTORY
No procedure history documented

IN-OFFICE MEDICATIONS AND/OR IMMUNIZATIONS
No qualifying data available.

MEDICATIONS
apixaban 5 mg oral tablet, 5 mg= 1 tab(s), Oral, BID, 3 refills
fluvastatin 20 mg oral capsule, 20 mg= 1 cap(s), Oral, every day at bedtime, 3 refills
hydroCHLOROthiazide 25 mg oral tablet, 25 mg= 1 tab(s), Oral, Daily, 3 refills
latanoprost 0.005% eye drops [2.5mL], See Instructions, 3 refills
Lidoderm 5% topical film, 1 patch(es), Topical, Daily, 3 refills
losartan 50 mg oral tablet, 50 mg= 1 tab(s), Oral, Daily, 3 refills
timolol maleate 0.5% eye drops [5mL], See Instructions, 3 refills
Voltaren 1% topical gel, See instructions, 6 refills

ALLERGIES
atorvastatin (Muscle Weakness)
simvastatin (Unknown)

SOCIAL HISTORY
Alcohol
Within past 12 months, Occasional Use, 11/26/2020
Substance Use
Never, 11/26/2020
Tobacco
Smoking tobacco use: Former smoker, quit more than 30 days ago. Type: Cigarettes., 01/14/2021

FAMILY HISTORY

0014C-David Grant Medical Center

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Office and Clinic Notes

Constitutional: no fever, no chills, no sweats, no weakness, normal appetite.

Respiratory: no shortness of breath, no cough, no wheezing.

Cardiovascular: no chest pain.

Gastrointestinal: no nausea, no vomiting, no constipation, no diarrhea, no abdominal pain.

Additional ROS info: Except as noted in the above Review of Symptoms and in the History of Present Illness and all over systems have been reviewed and are negative or noncontributory.

Pain
DVPRS Pain Scale: 0 - No pain (04/20/2026)
Past Week DVPRS score: 0 - No pain (04/20/2026)

PHYSICAL EXAM

Vitals & Measurements
T: 36.5 °C (Oral) **HR:** 100(Peripheral) **RR:** 16 **BP:** 128/69 **SpO2:** 97%
HT: 178 cm(Measured) **WT:** 88 kg(Measured) **BMI:** 27.77 kg/m2(Measured)

R Knee:

Inspection
Edema: N
Deformities: N
Ecchymosis: N
Erythema: N
Gait: Antalgic, short stance phase on rt

Palpation
Effusion: N
Temp: Nml
TTP medial joint line: N
TTP lateral joint line: N
TTP medial patellar facet: N
TTP lateral patellar facet: N
TTP quadriceps tendon: N
TTP patellar tendon: N
TTP tibial tuberosity: N
TTP medial tibial plateau: N
TTP lateral tibial plateau: N
TTP proximal anteromedial tibia: N
TTP medial femoral epicondyle: N
TTP lateral femoral epicondyle: N
Baker's cyst: N
Patellar Shrug: Neg
Patellar apprehension: Neg

Family history is negative

RECOMMENDATIONS
Health Maintenance
Pending (in the next year)
OverDue
Adult - Colorectal Cancer Screening Recommendation Gap due 10/05/17 One-time only
Adult - DoD PTSD Screening due 11/26/21 and every 1 years
Adult - Fall Risk Screening due 04/12/24 and every 1 years
Adult - Alcohol Use Screening due 06/06/24 and every 1 years
Due
Adult - Domestic Violence/Relationship Safety Screening due 04/21/26 Unknown Frequency
Adult - Food Insecurity Screening due 04/21/26 Unknown Frequency
Adult - Sexual Orientation due 04/21/26 Unknown Frequency
Adult - Tobacco Use Screening due 04/21/26 Unknown Frequency
Due In Future
CKD - Glomerular Filtration Rate not due until 12/15/26 and every 1 years
HTN - Serum Creatinine not due until 12/15/26 and every 1 years
CKD - Urine Albumin to Creatinine Ratio (uACR) not due until 12/16/26 and every 366 days
Adult - Depression Screening not due until 04/19/27 and every 1 years
Adult - Body Mass Index not due until 04/20/27 and every 366 days
Satisfied (in the past 1 year)
Satisfied
Adult - Body Mass Index on 04/20/26. Satisfied by AGUIRRE, SARA N
Adult - Depression Screening on 04/20/26. Satisfied by AGUIRRE, SARA N

CARE PATHWAYS THIS VISIT

No Results Found

Attachment(s): 4/21/2026 12:32 PDT (04/20/2026) Primary Care Note

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Office and Clinic Notes

ROM
Flexion: limited, physiological barrier met before end flexion
Extension: limitd, physiological barrier met before end extension
Catching: N
Locking: N
Crepitus: N

Strength/NV
Flexion: Nml
Extension: Nml
Distal NVI: Y

Special Tests
Cruciate ligaments
Lachman: Neg
Anterior drawer: Neg
Lever test: Neg
Posterior drawer: Neg

Collateral ligaments
Varus stress: Neg
Valgus stress: Neg

Meniscus
Thessaly: Neg

XR rt knee 12/2025

IMPRESSION:

Tricompartmental osteoarthrosis, most pronounced in the lateral compartment
where it is at least moderate.
Small knee joint effusion.

XR Hip radiographs b/l 9/2021

IMPRESSION: Severe bilateral hip osteoarthritis, not significantly changed since 9/27/2021 exam.

Electronically signed on: 04/21/2026 07:36 PDT

RYAN MARRS
DO